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Name: سماح محمد محمود عبد الجليل National ID: 27607053200161	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Patient Name : الأستاذ / سماح محمد محمود عبد الجليل	Request No. : 13005458
Patient No. : 27607053200161	Request Date : 15/03/2026
Path No. : 715/26	Verification Date : 18/03/2026
Sex : Female	Financial Category : جوماتاف روسي - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of specimen: Left modified radical mastectomy.

Gross Examination:

- * Left modified radical mastectomy specimen measured 22x20x5 cm, with covering skin ellipse measured 18x12 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 6x3x2 cm, located 2 cm from the least deep margin.
- * Axillary fat measured 8x6x2 cm, entangling 12 lymph nodes, the largest measured 1.5x1x0.5 cm.
- * 12 Blocks:
1:6- T.B.S., 7- N&A., 8- Deep margin., 9:112- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: No residual viable remnants of the previously diagnosed tumor.
- Cellularity (0%).
- * Angioinvasion: Absent.
- * Ductal carcinoma in situ: Absent.
- * Additional findings: Effect of therapy with microcalcifications.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Axillary lymph nodes: Twelve lymph nodes free of metastatic deposits, LNs (0/12).
- * Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:

- Left breast cancer (Post-NACT), Left modified radical mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor.
 - Ductal carcinoma in situ: Absent.

Verified by :
Dr.Duaa Mohamed

Printing Date : 11/04/2026 16:07:32

Page 1 of 2

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Age	: 49 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Nipple & areola: Free.
- Deep Margin: Free.
- Axillary lymph nodes: Twelve lymph nodes free of metastatic deposits, LNs (0/12).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0.

Signature:

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr.Duaa Mohamed

Printing Date : 11/04/2026 16:07:32

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Name: سماح محمد محمود عبد الجليل		GUSTAVE ROUSSY	
MR. : 5055			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
49 YRS , TPBC NACTH >> TCHP 25-1-2026 W 18 TAXOL CARBO MDT FOR SURGERY ECHO (28- 12-2025)>> EF 69 % PHEGO CYCLE 7 >> TO BE TAKEN 1-2-2026 >> PHARMACY patient today for c7 phesgo and waiting for surgery mdt POST NACT initially LT LABC clinically: LT breast residual edema + central distortion MDT: for LT MRM for c8 phesgo, finished NACT TCHP, pending surgery echo 28/12 69% PRE OP T4B N1 M0, NACT TCHP >> LT MRM >> YPT0 YPN0 0/12 > MDT 7/4/2026 >> ADJ PHESGO, PORT + FEMARA 7 YRS today for adj phesgo c8? + femara + supp A CASE OF BREAST CANCER ON ADJ HT FEMARA PT WILL START 1ST MONTH OF HT TODAY SO FOR FEMARA FOR 3 MONTH prem left breast cancer ct4b n1 mo ilc er6/8 pr8/8 her2 3+ ki 20 rec tchp on 15/3/2026 left mrm cr n0/12 port pending approval of phesgo pending for e2 fsh to be sure of menopausal status today for C1 phesigo 600/600 ECHO 18-5-2026 >> 65% zometa >> labs >> OK WT 83 E2 level >>less than 10 >> no need for zoladexpending PORT >> still pendig for c9 phesgo femara ca vit d methyltecno patch bec of pn ,e2 fsh postm for c11 phesgo + femara + supp tx, for echo next time for c12 phesgo + femara echo not done for new one before next cycle			
تحريراً في ,			
		التاريخ :	19-09-2026
		الوقت :	
		هنا أسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,			

mm/dd/yyyy



MDT FORM

Name:	Samah Mohamed Mohamoud Abed Elgalel	MRN:	5055
Age:	50 Y	ID:	
Menopausal Status:	POST	Phone:	01157247154
NRS SCORE:	PS 1	Marital Status:	MARRIED
BMI:	30	Family History:	+VE
CO Morbidity:	HTN	Reason Of MDT:	POST OP FOR ADJ TIT

Clinical Data:

BX: ILC G2

ER 6/8 **PR** 8/8 **HER** +3 **KI67** 20

Sonomammography:
PRE LT T4B N1 M0

Mets Workup:

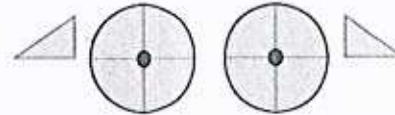
History:
POST NACT

Recommendations:
LT MRMPCR.....T0 N0.....LN 0/12

MDT Outcome & Plan

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

جمهورية مصر العربية
بمحافظة الجبل
سماح
محمد محمود عبدالجليل
١ مجاورة ٢٠
خمسة عشر مايو - القاهرة
٢٧٦.٧.٥٣٢.٠٠١٦١
KV8743869

٢٠٢٤/١١ ٢٧٦.٧.٥٣٢.٠٠١٦١
حاصله على دبلوم صنايع
انثى مسلمة
البطاقة سارية حتى ٢٠٣١/١١/١٦
١

4/7/26, 10:01 AM



MDT Form - Name: A.MDT - Name: E.MDT

إدارة رئيس الجمهورية
لحفظ صحة المرأة المصرية

04/07/2026

MDT FORM

Name:	Samiel Mohamed Mahmoud Abad Elgabal	HN:	5055
Age:	59 Y	ID:	27457553200161
Menopausal Status:	Post	Phone:	01157247154
NHS SCORE:	PS 1	Marital Status:	Married
BM:	22	Family History:	Yes
CO Morbidity:	None	Reason Of MDT:	Post OP FOR ADJITT

Clinical Data:

R

BN: UCG3
EBI: N/A
HER2: 3
X167: 20

Endometriography:
PVE AT 12-14 NO

Mata Workup:

History:

Post HACT

Recommendations:

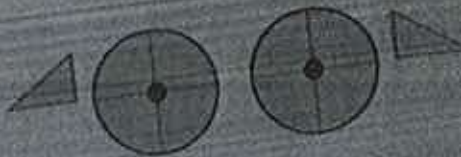
ET MDM ... PCW ... TO MD ... IN 0/17

MDT Outcome & Plan

For ADJIT

PCW ...

Schedule 7 years with consult every 6 mo



TNM

SIGNATURE		Revision	Psychiatric
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY

MDT FORM

1 / 9 / 2025 . G

Name	سماح محمد محمود	MRN	5.00
Age	49 y.	ID	5.00
Menopausal status	Pre-menopause	Phone	01157247154
NRS SCORE		Marital status	Divorced
BMI		Family history	+ve
CO morbidity	hypertension	REASON OF MDT	new Case

Clinical Data

BX: Invasive lobular Carcinoma

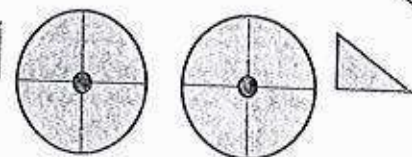
R

ER: 6/8 PR: 8/8 HER 2: 3+ Ki67: 20%.

Sonomammography:
- 10-2 o'clock locally advanced highly
suspicious process & multiple ipsilateral
multiple pathological L.N.

Mets Workup:

C-T Contrast → Free
Bone scan → Free



TNM
4 1 0
b

Recommendations: For NAT then MRM

MDT OUTCOME & PLAN - CSM
- MACT TCHP X 6
- 1st MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
M. Kall					

Patient Name : الأستاذ / سماح محمد محمود عبد الجليل
Patient No. : 27607053200161
Path No. : 2386/25
Sex : Female
Age : 49 Y
Referreing Doc :

Request No. : 13008277
Request Date : 28/07/2025
Verification Date : 03/08/2025
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast UOQ mass, BIRADs 5.
Nature of Specimen: Tru-cut biopsy, collected on 27/07/2025.

Gross Examination:

- Three grey white tissue cores, the longest was 1.5 cm. Totally submitted.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive lobular carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * DCIS: Absent.
- * LCIS: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Microcalcification: Present.

* IPT (Positive control showed appropriate reactivity):

- ER: Strong nuclear reaction in 30% of tumor cells (score 6/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 20%.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Verified by :
Dr.Duaa Mohamed

الطبيب
د. دينا محمد

Printing Date : 03/08/2025 12:59:32

Page 1 of 3

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Patient Name : الأستاذ / سماح محمد محمود عبد الجليل
Patient No. : 27607053200161
Path No. : 2386/25
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13008277
Request Date : 28/07/2025
Verification Date : 03/08/2025
Financial Category : نفقة الدولة - جوسفاروسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

- Invasive lobular carcinoma, Grade 2.
IPT;
- ER: Positive (score 6/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67%: 20%.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

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Page 2 of 3

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Patient Name: Samah Mohamed Mahmoud Abd Al Galeel	Date : 03/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
Medical ID: 27607053200161	Pat. ID: 110815
7778	

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 49-year-old female with left breast lump of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Upper central posterior third circumscribed oval equal density mass lesion.
- No speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated 1 o'clock, posterior third, circumscribed oval parallel homogenous hypoechoic benign looking solid mass lesion measuring 11 x 6mm.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Upper central, middle & anterior thirds, global asymmetry with surrounding distortion and no suspicious microcalcifications.
- Thickened skin and coarsen trabeculae.
- **Ultrasound** demonstrated 10-2 o'clock, middle & anterior thirds, 4 mm deep to the skin, area of altered parenchyma mounting to mass formation with distortion measuring 7 x 4 cm. anterior ductal extension is noted reaching the nipple that is seen retracted.
- Thickened skin reaching 6 mm & coarsen trabeculae.
- More than three pathological axillary LNs are noted showing eccentric hila and thickened cortices reaching 5 mm. no pathological infra or supra-clavicular lymph nodes.

Patient Name: Samah Mohamed Mahmoud Abd.Al Galeel	Date : 10/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 49 Year(s)
Medical ID: 2022/2025	Pat. ID: 110815

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- No sizable pulmonary nodules, masses, areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- Increased cardiac size.

II Abdomen & pelvis:

- The liver is of average size showing regular outlines. It showed no focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average spleen with no focal lesions
- Normal CT appearance of the pancreas, kidneys, suprarenals glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathologically enlarged abdominal lymph nodes..
- no ascites.

Opinion:

- Reported case of left breast cancer revealed:
- Unremarkable Ct scan of the chest, abdomen and pelvis

*Best regards,
Dr. Asser Ahmed, Radio-diagnosis consultant*

14/08/2025 2:40 AM



15 May Hospital



Name	Samah Mohammed
Date	Jul. 14, 2025
Time	12:46 PM

Digitalized Low Dose Soft Tissue Mammography With Complementary Breast US Examination Of Both Breasts Revealed:

- ACR B : Mixed fibro-fatty breast parenchyma.
- An ill-defined hyperdense lesion is seen at the upper quadrant of the left breast corresponding by US to an ill-defined shaped heterogeneous hypoechoic lesion is seen at 12 o'clock surrounded by minimal edema and increased skin thickness and casting posterior shadowing. It measures 2.5x2.4 cm
- Few enlarged left axillary LNs showing increased cortical thickness (>3mm) and effaced fatty hilum. The largest measures 1.3 cm with faint hilum.
- No pathological micro calcification clusters.
- No right breast solid or cystic lesions.
- Normal central ducts system.
- Normal skin thickness and contour as well as nipple areola complexes.
- Right nonspecific axillary lymph nodes showing preserved fatty hila.

Opinion:

- Bilateral breast fibroadenosis with Left breast upper quadrant suspicious lesion associated with ipsilateral axillary lymphadenopathy (BIRADS IVb) for pathological correlation.

Best Regi
Dr. Eman You

BIRADS scoring is the breast imaging and reporting data system for classifying the radiological diagnosis of breast lesions:

- BIRADS 0: Needs further investigations.
BIRADS 1: Normal findings.
BIRADS 2: Benign.
BIRADS 3: Probably benign.
BIRADS 4: Probably malignant.
BIRADS 5: Malignant.
BIRADS 6: Proven pathologically malignant

Patient Name: Samah Mohamed Mahmoud Abd Al Galeel	Date : 03/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
7778 Medical ID: 27607053200161	Pat. ID: 110815

Conclusion:-

Right breast (BIRADS 3): UIQ probably benign solid mass lesion mostly fibroadenoma.

Left breast (BIRADS 5): 10-2 o'clock locally advanced highly suspicious process with ipsilateral multiple pathological level 1 axillary lymph nodes

Best Regards,
Dr. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations

BIRADS 0: Non-informative study.

BIRADS 1: Normal study, routine screening

BIRADS 2: Benign findings, routine screening

BIRADS 3: Probably benign, for close follow up.

BIRADS 4: Probably malignant, for biopsy.

A: mild suspicion.

B: moderate suspicion.

C: high suspicion.

BIRADS 5: Malignant, for biopsy.

BIRADS 6: Pathologically proven malignant

Suspected Cancer Risk

0 %

0 %

2 %

50 %

98-100%

Signature Dr. Amal Hegazy
PM 12:00 2025/08/03

Name : Samah Mohammed Mahmoud Abdel
Jalil

Date : 16-08-2025 10:29

Branch : Dokki

Medical No. : 177719

Ref.Physician Prof : Gostaf Roseh Hospital

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 49-year-old lady, with left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen all over the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine